

LECTURE 1 — PERITONEUM, MESENTERY & RETROPERITONEUM (Table)

Bold = high-yield.

PERITONEUM & PERITONITIS	
Topic	Key points
Pain	Parietal = localized somatic; visceral = poorly localized midline
Peritonitis causes	Bacterial, chemical (bile/barium), allergic (starch), traumatic, ischaemic, FMF
Localized peritonitis	Somatic pain, guarding + rebound = peritonism ; diaphragm → shoulder tip (C5)
Diffuse peritonitis	Life-threatening (perforated viscus); Hippocratic facies, lies still, board-like rigidity, septic shock
Investigation/Mgmt	CT = choice ; erect CXR (subdiaphragmatic gas); source control + lavage + broad-spectrum antibiotics + NG + fluids

SPECIFIC PERITONITIS & ASCITES	
Type	Key points
SBP	Cirrhosis/ascites; paracentesis neutrophils >250/mm³ (culture –ve 60%); cefotaxime
Biliary	Post-cholecystectomy (clip slip); drain + ERCP stent / surgery
TB peritonitis	Wet ascitic (90%) vs dry/plastic; exudate, ADA high sens/spec ; anti-TB
Fitz-Hugh-Curtis	Perihepatitis (Chlamydia/gonococcus), Glisson's capsule scarring
Ascites	>1.5L clinical; shifting dullness. Transudate <25 (cirrhosis/CCF); exudate >25 (malignancy/TB/Budd-Chiari/Meigs') ; spironolactone+furosemide, TIPS, paracentesis
Carcinomatosis	Omental cake ; HIPEC for pseudomyxoma
Abscess	Swinging pyrexia; CT dx + drainage; <5cm antibiotics, >5cm drain

MESENTERY & RETROPERITONEUM	
Condition	Key points
Mesenteric adenitis	Ileocaecal nodes (viral/Yersinia); RIF pain + fever + shifting tenderness (vs appendicitis)
Acute mesenteric ischaemia	SMA/coeliac embolism; pain out of proportion ; Drummond marginal artery
Chronic mesenteric ischaemia	Postprandial pain + fear of eating + weight loss ; CT angiography
Mesenteric cysts	Chylolymphatic (commonest, enucleate); enterogenous (resect bowel)
Psoas abscess	Crohn's (or TB spine Pott's) ; groin swelling, hip extension pain; CT drainage
Retroperitoneal sarcoma	Late + large; liposarcoma/leiomyosarcoma; surgical resection (chemo/RT poor)

LECTURE 2 — INTESTINAL OBSTRUCTION (Table)

Bold = high-yield.

CLASSIFICATION & CAUSES	
Type	Examples
Dynamic (mechanical)	Peristalsis vs block
Adynamic	Paralytic ileus, pseudo-obstruction
Intraluminal	Faecal impaction, bezoar, gallstone ileus
Intramural	Stricture (Crohn's/TB), malignancy , intussusception, volvulus
Extramural	Adhesions (commonest SBO), hernia

PATHOPHYSIOLOGY & STRANGULATION	
Topic	Key points
Distension	Gas (N■ 90%) + fluid (up to 2.5L/24h retained)

Topic	Key points
Systemic	Dehydration, ↑urea + haematocrit; hypokalaemia NOT in simple obstruction
Strangulation	Venous before arterial ; direct pressure (hernia/band), interrupted flow (volvulus/intussusception), closed loop. Constant pain (not opiate-controlled) + peritonism + shock
Closed loop	Competent ileocaecal valve + colonic obstruction → perforation risk
Intussusception	Children 5–10mo idiopathic; adults = lead point (tumour) ; ileocolic; target sign
Volvulus	Sigmoid commonest (adults) ; obstruct >180°, vascular >360°; decompress per anum

CLINICAL, IMAGING & MANAGEMENT

Topic	Key points
Cardinal quartet	Pain, vomiting, distension, absolute constipation. High SBO = early vomit, minimal distension; LBO = late vomit, early distension
Constipation exceptions	Richter's hernia, gallstone ileus, partial (diarrhoea)
Imaging	SBO = central, valvulae conniventes (ladder); LBO = peripheral, haustra. CT accurate; strangulation = clinical dx. Water-soluble contrast predicts SBO resolution; barium contraindicated
Management	Resuscitate first (NG + fluids) UNLESS strangulation/closed loop. Early surgery: hernia, strangulation, virgin abdomen. Adhesive → conservative ≤72h. Viability = colour/sheen/peristalsis/pulsation after hot packs 10min